INVESTMENT COMMITTEE MEMO | DISCUSSION DRAFT

A measured path into healthcare workflow AI

Decision date: 10 October 2026 | Subject: Silna Health and provider-side administrative automation | Prepared using public sources and analyst assumptions.

RECOMMENDATION: Advance structured diligence on Silna, benchmark Tennr and Rhyme, validate current Notable coverage, and do not authorize an equity investment or production deployment yet.

Investment question

Can a differentiated authorization and referral automation vendor deliver measurable patient-access and administrative benefits for a health system while also offering an attractive entry-priced venture equity opportunity? Treat the operational and financial-investor hypotheses as separate gates.

Decision summary

Decision track	Preliminary conclusion	Evidence needed before commitment
Commercial technology	Permit a tightly controlled design-phase pilot if incumbent gap and safety review are confirmed.	Aggregate workflow baselines, vendor quote, exception rates, quality guardrails and security approval.
Equity investment	No investment approval on public information alone. Silna remains the first candidate to diligence.	Actual ARR, gross margin, retention, sales efficiency, burn, cap table and signed entry terms.

Why now

CMS-0057-F introduces certain process obligations generally from 2026 and payer FHIR API requirements generally from 2027, largely for non-drug prior authorizations. This improves the rationale for digitization without guaranteeing demand for any vendor [S04, S35]. U.S. digital health raised $7.4 billion across 244 deals in H1 2026, but financing was concentrated in large rounds [S32].

Why this case is strategically specific

Intermountain publicly announced a Notable partnership in 2020, covering intake and registration automation [S31]. Later vendor case studies cite further intake and pediatric workflows [S33, S34]. The present contractual scope is unknown. A new product must clear an incremental-value test against existing tools, not merely beat a manual baseline.

Investor discipline

The proposed model deliberately permits a negative first-year operating return and a less-than-1x downside venture outcome. A compelling product demonstration is not an investable term sheet.

SECTOR AND STRATEGIC FIT

A narrow, testable thesis

The opportunity is workflow reliability and measurable care access, not a claim that AI alone can eliminate all administrative expense.

The serviceable wedge

Begin with tightly bounded provider-side administrative workflows, such as receiving referral packets, verifying eligibility, assembling prior authorization documentation, flagging missing information, routing exceptions, and tracking completion. Keep medication authorization, payer-side processing and complex clinical judgment separate, because workflows, regulatory rules, vendor buyer types and safety exposure differ [S04, S10].

Competitive map

Candidate	Comparative rationale	Material disconfirming evidence
Silna Health	Earlier documented seed and Series A funding totaling $27m announced in March 2025; focused care-readiness workflows [S10].	Can it integrate and perform at multi-specialty health-system scale? Is an investable round available?
Tennr	Referral orchestration and specialized document processing. June 2025 Series C $101m at reported $605m valuation [S08].	Does price, contract structure and existing scale justify an equity return at a new entry valuation?
Rhyme	Payer/provider connectivity and authorization network benchmark [S14].	Actual payer coverage, automation denominator and exception quality in relevant cohorts.
Notable / existing stack	Historical Intermountain relationship, and later vendor-reported program examples [S31, S34].	Is the relationship active and does an incumbent module eliminate incremental benefits?

Market sizing method

Do not manufacture a TAM from national administrative burden or aggregate transaction spend. A credible bottom-up SAM requires verified buyer counts by provider type, share of workflows in-scope, evidence-based contract ACV, vendor adoption constraints, and exclusions for incumbent-covered cases. Those inputs remain unknown. The workbook supplies a blank driver-based market framework rather than a false precise dollar estimate.

Differentiation and defensibility diligence

Verify payer relationships, integrations, hard-to-replicate task data, data ownership and access rights, observable accuracy and exception handling, customer concentration, and switching costs. A generic LLM wrapper is insufficient evidence of durable moat. Request blinded, de-identified test samples and cross-specialty customer references.

HOSPITAL OPERATIONS

Pilot economics are conservative by design

The following numerical inputs are fully illustrative. They are not Intermountain forecasts, employee costs, vendor proposals or observed vendor performance.

Metric	Downside	Base	Upside
Pilot cases per month	1,200	2,000	3,000
90-day net cash	($66,636)	($39,834)	($31,610)
Scaled annual cash benefit	$16,416	$148,176	$486,000
Annual recurring costs	$166,800	$122,400	$138,600
Scaled steady-state net cash	($150,384)	$25,776	$347,400
First scaled year net cash	($196,950)	($31,268)	$249,500

Key base-case underwriting observations

At hypothetical 12,000 monthly in-scope cases, 70% eligibility, 60% successful automation, 7 net minutes recovered per case and a $42 loaded hourly proxy, annual staff-capacity value is $296,352. Only a separately assumed 50% is modeled as realized cash. That produces $148,176 of recurring annual benefit against $122,400 recurring expense, or 1.21x coverage.

The cash realization threshold for operating break-even is 41.3%. At a proposed 1.20x annual recurring benefit-to-cost hurdle, the maximum base monthly vendor fee is approximately $8,090, versus the illustrative fee of $8,000. The narrow headroom makes pricing diligence essential.

Why the 90-day pilot loses cash

A 90-day pilot uses a smaller monthly cohort, a three-stage ramp and $30,000 in setup costs. Base net pilot cash is ($39,834). This is a planned evidence-gathering expenditure, not proof of vendor failure. The first fully scaled year also remains cash negative at ($31,268) because its benefits ramp but subscription and rollout costs are incurred.

Measurement architecture

Use a retrospective baseline and an appropriately matched comparison group. Measure completion within SLA, time to verified authorization, staff touch minutes including rework, denials and appeals, patient access, data-extraction error rate, manual escalations, security incidents, overtime, contractor spending and avoided planned hiring. Define clinical stop rules before go-live; avoid making the financial model responsible for patient-safety judgments.

VENTURE INVESTMENT

Reverse underwriting, not a fabricated valuation

Equity inputs are hypothetical and do not imply an available Silna financing, company revenue, valuation or fund check size.

Transaction and exit driver	Downside	Base	Upside
Pre-money ($mm)	$110.0m	$80.0m	$60.0m
Round size ($mm)	$30.0m	$25.0m	$20.0m
Post-money ($mm)	$140.0m	$105.0m	$80.0m
Initial ownership	1.4%	1.9%	2.5%
Exit ARR ($mm)	$30.0m	$80.0m	$160.0m
Exit EV / ARR	3.0x	6.0x	9.0x
Gross MOIC	0.32x	2.97x	13.50x
Gross six-year IRR	-17.2%	19.9%	54.3%

Interpretation and required evidence

In the illustrative base scenario, a $2m primary check into an $80m pre-money / $25m raise buys 1.9% initial ownership. Following 35% cumulative dilution, it retains 1.2%. At $80m hypothetical exit ARR and 6.0x EV/ARR, investor proceeds are $5.9m, equal to 2.97x gross MOIC and 19.9% annualized gross IRR over six years.

To achieve a full 3.0x MOIC with the same assumed entry, dilution and 6.0x exit multiple, the company would need approximately $80.8m of exit ARR, assuming a 100% enterprise-to-equity conversion and no preferential exit waterfall. The calculation is a hurdle test, not an ARR forecast.

Why public financing is not a valuation comp

The $605m Tennr June 2025 figure is a reported financing snapshot, not an independently observed October 2026 enterprise value and not a revenue multiple [S08]. Silna reported $27m combined seed and Series A funding, but cumulative capital raised is not a disclosed market valuation [S10]. As a result, 3x / 6x / 9x EV/ARR assumptions are sensitivity levers rather than empirically derived comparable medians. Real deal terms and audited financials must replace them before any capital authorization.

Critical omissions

No preferred liquidation waterfall, participation, founder vesting, convertible notes, net debt, transaction fees, follow-on checks, cash distributions, option-pool reshaping or GP carried interest are modeled. All would matter for an investment committee-quality actual term sheet assessment.

EXECUTION AND GOVERNANCE

Decision gates and 90-day diligence

Any failure in clinical safety or contractual compliance supersedes a positive financial result.

Gate	Test	Decision consequence
1. Incumbent-first review	Document current Notable and other tools, incremental pricing and contract scope.	If capability already exists at lower incremental cost, prioritize incumbent evaluation.
2. Controlled pilot eligibility	Approve de-identified aggregate baseline, BAA, security, clinical oversight and monitoring plan.	No production deployment without approved controls.
3. Operating evidence	Validate touch time, clinical exceptions, quality, payer mix, and cost-accounting realization.	Do not scale unless verified recurring cash benefit meets 1.20x recurring cost with safe care.
4. Venture quality	Confirm gross margin, NRR, ARR growth, burn, pipeline conversion and customer references.	No equity commitment without primary diligence.
5. Entry terms and rights	Confirm a real financing, governance, preferred terms, cap table and downside economics.	Decline or renegotiate uninvestable pricing, even if pilot succeeds.

Suggested implementation workplan

Window	Activities	Output
Weeks 1-2	Confirm incumbent gap; map eligible workflows, payer mix, comparator design, quality guardrails, privacy and security.	Pilot protocol and locked baseline.
Weeks 3-6	Run constrained workflow in selected specialties, capture audit-trail cases, collect control-group metrics.	Interim safety and exception review.
Weeks 7-10	Audit completion and rework by subgroup; compare vendor and incumbent costs; validate realized cash levers.	Measured economics and quality report.
Weeks 11-13	Conclude: scale, modify, retain incumbent or terminate. Run investment diligence separately.	Go / no-go memo and updated models.

Final recommendation

Maintain a diligence-first stance. Do not make an equity recommendation from public funding headlines or an illustrative ROI model. The next value-creating step is a documented incumbent comparison and a bounded pilot protocol, while soliciting verified financial and transaction information from the preferred startup.

METHODS AND EVIDENCE

Source notes and limitations

Selected directly relevant evidence. Full 36-source register, dates, URLs and qualifications appear in the accompanying Excel workbook.

S01 | Intermountain Health (Undated / accessed 2026-10-10). Six investment sectors; early and growth stage; piloting and post-investment support. https://intermountainhealthcare.org/about/ventures

S04 | CMS (2024-01-17). Prior auth operational provisions 2026 and API requirements mostly 2027; principally non-drug services. https://www.cms.gov/newsroom/fact-sheets/cms-interoperability-prior-authorization-final-rule-cms-0057-f

S06 | American Medical Association (2026-05-13). 2025 physician survey: 40 PAs per physician each week and 13 hours per practice per week. https://www.ama-assn.org/press-center/ama-press-releases/ama-survey-prior-authorization-reform-pledge-falls-short-physicians

S08 | Fortune (2025-06-18). Tennr $101M Series C at reported $605M valuation. https://fortune.com/2025/06/18/tennr-health-tech-ai-patient-referral-ivp-a16z-lightspeed-iconiq-series-c/

S09 | Tennr (2025-06-18). Tennr raises $101M Series C and introduces referral network. https://www.prnewswire.com/news-releases/healthcare-referrals-are-where-patients-get-lost-tennr-raises-101m-to-bring-the-visibility-our-system-desperately-needs-302485255.html

S10 | Silna Health (2025-03-25). Silna launched with $27M cumulative seed and Series A funding. https://www.silnahealth.com/resources/introducing-silna-health/

S11 | Silna Health (2026-01-28). Predictive document intelligence; reported support for >150,000 patients. https://www.silnahealth.com/resources/predictive-document-intelligence/

S14 | Rhyme (Undated / accessed 2026-10-10). Provider-payer authorization network and touchless workflows. https://getrhyme.com/

S31 | Intermountain Health (2020-10-06). Intermountain announced strategic partnership with Notable for automated patient registration, insurance eligibility and intake, with reported reduction in check-in time. https://news.intermountainhealth.org/intermountain-healthcare-partners-with-notable-health-to-expand-the-my-health-digital-platform-for-patients/

S32 | Rock Health via Healthcare Dive (2026-07-16). H1 2026 US digital health raised $7.4B across 244 deals; financing concentrated in megadeals. https://www.healthcaredive.com/news/digital-health-funding-first-half-2026-rock-health/825215/

S33 | Notable (2025-06-09). Vendor-reported Intermountain digital intake outcomes. https://www.notablehealth.com/blog/how-automation-improves-patient-outcomes-at-scale

S34 | Notable (Undated, accessed 2026-10-10). Intermountain pediatric pre-visit forms case study; 12,500 forms over 3 months. https://www.notablehealth.com/customer-stories/intermountain-healthcare-drives-77-digital-completion-rate-of-well-child-forms-through-automation

S35 | CMS (2026-05-13). Electronic prior authorization acceleration initiative and early adopters. https://www.cms.gov/newsroom/press-releases/cms-announces-early-adopters-advance-solutions-electronic-prior-authorization-accelerating-momentum

S36 | Notable (2026-01-07). Inova partnership specifically involves revenue cycle referral management and patient access. https://www.notablehealth.com/blog/inova-health-partners-with-notable-to-support-system-wide-ai-and-digital-transformation-strategy

Methods disclosure

All analyst scenario amounts, financial forecasts, cash conversion factors, assumed vendor prices, transaction valuation inputs, forward ARR, dilution and exit multiples are explicitly illustrative and are not represented as public company facts. Historical financing figures and historical customer outcomes are time stamped. Certain historical Notable outcomes are vendor-reported. Source quality does not replace first-party operational validation.

Decision authority

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